

COGNITIVE BEHAVIORAL THERAPY (CBT) – SESSION 3

New Case: Rohan (26 y/o) – Contamination OCD + Checking

Highly Detailed – Realistic Style

SESSION INFORMATION

Client:	Rohan (26 years, Male, Software Engineer, Bangalore)	Therapist:	Dr. A.
Session #:	3 of 18–20	Focus:	Coping Strategies + Building Motivation for ERP

PRE-SESSION THERAPIST NOTES

Client arrived looking tired but slightly more engaged than last session. He completed monitoring on 6 out of 7 days and attempted to delay compulsions on a few occasions (mostly when anxiety was moderate). He reported that delaying by 1–2 minutes was sometimes possible but very difficult when anxiety was high. He continues to wash hands excessively and check switches/locks. Today's plan: Review homework in detail, introduce cognitive defusion / labeling technique, continue building motivation for ERP by exploring the cost of OCD, and assign new homework. Maintain gentle, non-pressuring pace.

SESSION START – HOMEWORK REVIEW

Therapist: Good morning, Rohan. How has your week been? I see you brought the notes again. How did the delaying practice go?

Client: (speaking in a low, tired voice) It was mixed. On some days when the urge wasn't too strong, I was able to wait 1–2 minutes before washing. I tried using breathing during that time. But when the anxiety was very high — like after touching something in the office or outside — I couldn't stop myself at all. I still washed many times. I also checked the switches at night 4–5 times almost every day. I feel like I'm not making much progress.

Therapist: Thank you for trying, even on the days it felt difficult. It's very normal that delaying is harder when anxiety is high — that's exactly why we start small and build up gradually. The fact that you were able to delay sometimes shows that the urge is not completely uncontrollable. What happened to your anxiety during the times you were able to wait those 1–2 minutes?

Client: It went up at first, but then it came down a little by itself. Not completely, but enough that I could wash "normally" instead of 10–15 times. That felt a bit surprising.

INTRODUCING COGNITIVE DEFUSION / LABELING

Therapist: That observation is very important — the anxiety rose but then started to come down on its own when you didn't immediately do the compulsion. This is something we call **habituation** — anxiety naturally decreases over time if we don't keep feeding it with compulsions. To help with this, I'd like to introduce another small skill called **Labeling** or **Cognitive Defusion**. Instead of getting pulled into the thought ("I might be contaminated"), you can label it as an OCD thought. For example: "I'm having the thought that I might be contaminated" or simply "This is an OCD thought." This creates a little distance from the thought instead of treating it as fact. Would you like to try this?

Client: I can try. Sometimes when the thought comes, I already know it's probably OCD, but it still feels very real and scary.

Therapist: That's completely normal. The goal of labeling isn't to make the thought disappear or to convince yourself it's false. It's to create a small gap between you and the thought so you have a bit more choice in how you respond. We can practice it together now if you'd like.

Client: Okay. I'll try saying it in my mind when the thought comes: "This is an OCD contamination thought."

EXPLORING THE COST OF OCD (BUILDING MOTIVATION)

Therapist: You mentioned last time that you want to spend less time washing and checking, and feel less exhausted and ashamed. Can we look a bit more at what OCD is currently costing you in your daily life? This can help us both understand how important change is for you.

Client: It takes a lot of time. Sometimes 30–40 minutes just on washing in a day. I avoid shaking hands with colleagues or touching things in meetings. I've stopped going out with friends because I'm scared of public places. I feel tired all the time and ashamed that I can't control this. I also feel like I'm becoming more isolated.

Therapist: That's a significant cost — time, energy, social life, self-esteem, and connection with others. When you look at this list, how important does it feel to you to work on reducing these compulsions, even if it's difficult?

Client: It feels important. I don't want to keep living like this. But I'm still scared of what will happen if I stop washing and checking.

Therapist: That fear is very understandable. We'll address it gradually and safely. The fact that you can clearly see the cost of OCD is a strong reason to keep moving forward, even when it feels scary.

HOMEWORK ASSIGNMENT

Therapist: For this week:

1. Continue monitoring washing and checking
2. Practice labeling: When the contamination thought comes, gently say in your mind "This is an OCD thought" and notice what happens
3. Continue trying to delay compulsions by 1–2 minutes when possible, using breathing or labeling during that time

Client: I'll try the labeling one. It feels a bit easier than fully stopping the compulsion.

Therapist: That's perfect. We build skills step by step.

SESSION SUMMARY & FEEDBACK

Therapist: Today we reviewed your attempts at delaying, introduced labeling as a new tool, and looked at the real cost OCD is having on your life. How are you feeling about today's session?

Client: It was okay. Seeing the cost written down made me feel more motivated, but I'm still scared of reducing the compulsions. The labeling idea feels manageable.

Therapist: That's completely fair. Any questions before we finish?

Client: Not really.

POST-SESSION THERAPIST NOTES

- Client showed partial success with delaying compulsions and good engagement with new labeling technique.
- Clear motivation emerging from recognizing the cost of OCD on daily life and relationships.
- Client remains appropriately cautious about fully reducing compulsions.
- Next session: Review labeling and delaying practice, continue building motivation, begin discussing ERP hierarchy in more detail if client shows readiness, introduce more defusion or mindfulness techniques if helpful.

Disclaimer: This is a completely fictional but realistic simulation created solely for educational and AI training purposes. It does not represent any real person or therapy.